

Prior Authorization Policy

Document ID	RHN-REV-021	Version	2.4
Effective	2026-04-01	Review by	2027-04-01
Owner	Revenue Cycle, Utilization Management		
Approved by	Chief Financial Officer and Chief Medical Officer		
Applies to	All Riverside Health Network facilities and ordering providers		

1. Purpose

Prior authorization (PA) is the payer's approval, obtained before a service is delivered, that the service is covered and medically necessary. A claim for a service that needed approval and did not have it is denied, and the denial code is "Prior authorization missing". This policy lists the services that need approval, the time a payer has to decide, and who at Riverside Health Network does what.

2. Scope

The policy applies to services ordered for patients covered by Medicare Advantage (Northstar Advantage), Medicaid, Evergreen Health Plan and Cascade Mutual. Original Medicare does not use prior authorization for most services on this list, and self-pay patients are quoted a price instead. The Utilization Management team keeps the payer-specific exceptions in the payer portal matrix; where a payer's contract differs from this policy, the contract controls.

3. Services that require prior authorization

The decision turnaround is counted in business days from the time the payer receives a complete request. An expedited request is for a patient whose health or ability to regain function would be put at serious risk by the standard wait; the ordering provider must attest to this on the request.

Service category	Examples	Prior authorization	Standard decision	Expedited decision	Submit at least
Advanced imaging	MRI, CT, PET scans, cardiac MRI	Required	5 business days	24 hours	5 business days before the scan
Elective inpatient admission	Planned surgery, scheduled medical admission	Required	3 business days	24 hours	7 business days before admission
Outpatient surgery	Ambulatory surgery and endoscopic procedures	Required	5 business days	48 hours	7 business days before the procedure
Cardiac rehabilitation	Phase II supervised exercise program	Required	7 business days	72 hours	10 business days before the first session
Home health services	Skilled nursing and therapy visits after discharge	Required	2 business days	Same day	At discharge planning, before the patient leaves
Durable medical equipment over \$500	Home oxygen, hospital bed, powered mobility	Required	3 business days	24 hours	5 business days before delivery

Service category	Examples	Prior authorization	Standard decision	Expedited decision	Submit at least
Specialty infusion drugs	Biologic and high-cost infused therapies	Required	3 business days	24 hours	5 business days before the first dose
Sleep studies	Overnight polysomnography	Required	5 business days	Not available	7 business days before the study
Physical therapy beyond 12 visits	Continuing therapy after the initial 12 visits	Required	3 business days	Not available	3 business days before visit 13
Emergency admission through the ED	Any admission that begins in an emergency department	Not required; notify the payer	Not applicable	Not applicable	Within 24 hours of admission
Office visits, laboratory tests, X-ray, ECG, echocardiogram	Primary care and specialist visits, routine tests	Not required	Not applicable	Not applicable	Not applicable

4. How to request authorization

1. The ordering provider documents the clinical reason in the order. The order is not released to scheduling until the authorization status is "approved" or "not required".
2. The Utilization Management coordinator submits the request through the payer's portal or by fax on the same day the order is signed, attaching the clinical notes the payer's criteria require.
3. The coordinator records the reference number and the decision due date in the electronic record and checks the status daily until a decision arrives.
4. If the payer asks for more information, the coordinator returns it within 2 business days. The payer's clock restarts when the information arrives.
5. If the request is denied, the ordering provider decides within 2 business days whether to appeal or to change the order. A peer-to-peer call is requested for every denial of an advanced imaging or elective inpatient request.

5. Expedited requests

A request may be marked expedited when the ordering provider attests that the standard wait could seriously jeopardize the patient's life, health or ability to regain maximum function. Examples are a cardiac MRI for a patient with new symptoms and a rising troponin, or home oxygen for a patient who cannot be discharged without it. The coordinator calls the payer's utilization line after submitting and notes the call in the record.

6. When authorization is missing

If a service was delivered without a required authorization the coordinator asks the payer for a retrospective authorization within 14 days of the service. Where the service was an emergency, the emergency notification rule in section 3 applies and no authorization is needed. Claims that are denied for "Prior authorization missing" are sent to Denials Management and are written off only after the appeal window in the payer's contract has closed.

7. Monitoring

Utilization Management reports monthly to the Revenue Cycle Committee: requests submitted, the share decided within the turnaround in section 3, expedited requests, and denials by reason. A payer that exceeds its turnaround on more than 10% of requests in a quarter is raised with the payer's contract manager.

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